

August 12, 2026

TRICARE East (Humana Military)
Provider Appeals Department

RE: Formal Appeal of Claim Denial — Lavern Kassulke (MRN MUSC7509202), Claim MUSC-20250620-6184-1

Patient	Lavern Kassulke (DOB 1950-08-03)
MRN	MUSC7509202
Member ID / Group	TRI504985943 / GRP-87929
Claim number	MUSC-20250620-6184-1
Date of service	2025-06-20
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	I25.10 — Atherosclerotic heart disease of native coronary artery
Denied amount	\$993.65
Denial code	CARC 11 / RARC M64 — The diagnosis is inconsistent with the procedure.
Appeal deadline	2025-10-21

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health is submitting this formal first-level appeal on behalf of patient Lavern Kassulke (MRN: MUSC7509202, Member ID: TRI504985943) regarding the denial of Claim MUSC-20250620-6184-1 for services rendered on June 20, 2025, at MUSC Health Rutledge Tower Ambulatory Care by James H. Ferrell, MD (NPI 1992837465). TRICARE East (Humana Military) denied the full billed amount of \$993.65 under CARC 11 ("The diagnosis is inconsistent with the procedure") and RARC M64 ("Missing/incomplete/invalid other diagnosis"), with the payer remark stating: "The diagnosis code reported does not support the procedure billed." MUSC Health respectfully disagrees with this determination. The diagnosis codes submitted are clinically accurate, mutually supportive, and fully consistent with the level of service billed. We request reversal of the denial and payment at the allowed amount of \$695.14.

CLINICAL BACKGROUND

Mr. Kassulke is a 76-year-old male with an extensive and well-documented chronic disease burden. His active problem list includes atherosclerotic heart disease of native coronary artery (I25.10), which has been a recorded condition since 2004, and for which he has been maintained on furosemide injection, Lasix 40 mg, and metoprolol succinate extended-release (Toprol) — a medication regimen consistent with ongoing cardiovascular management. He additionally carries active diagnoses of hypertension (I10, onset 1969), prediabetes (E11.9), anemia (D64.9), overlapping malignant neoplasm of colon (C80.1), recurrent rectal polyp, and localized primary osteoarthritis of the hand (M17.9). His history includes multiple courses of combined chemotherapy and radiation therapy. This constellation of conditions represents a high-complexity, multi-system patient requiring coordinated, medically necessary outpatient management. The June 20, 2025, encounter was billed as an established outpatient visit of moderate complexity (CPT 99214) with I25.10 as the primary diagnosis and M17.9 as a secondary diagnosis.

BASIS FOR APPEAL

The denial under CARC 11 and RARC M64 is not supported by the clinical record. CPT 99214 describes an office or outpatient visit for an established patient requiring moderate medical decision-making or at least 30 minutes of total time. This level of service is entirely appropriate and well-supported for a 76-year-old patient with active atherosclerotic heart disease managed with multiple cardiac medications, in the context of co-existing hypertension, prediabetes, anemia, and a history of malignancy. ICD-10 code I25.10 — atherosclerotic heart disease of native coronary artery without angina pectoris — is a recognized, billable diagnosis code that directly describes a chronic condition requiring ongoing physician evaluation and medication management. It is not a symptom code, an unspecified code, or an incidental finding; it is a primary chronic condition actively treated with documented medications. The secondary diagnosis M17.9, osteoarthritis unspecified, reflects an additional active condition also under management with naproxen sodium. The assertion that these diagnoses are inconsistent with an established-patient evaluation and management service is without clinical or coding basis. CPT 99214 is a broadly applicable E/M code; no procedural or diagnostic exclusion exists that would render I25.10 or M17.9 incompatible with its use. The denial appears to reflect either a claims-processing error or an overly restrictive automated edit that did not account for the full clinical picture documented in the record.

REQUESTED ACTION

MUSC Health respectfully requests that TRICARE East (Humana Military) conduct a complete clinical and coding review of Claim MUSC-20250620-6184-1, reverse the denial in full, and issue payment at the previously determined allowed amount of \$695.14 for the services rendered on June 20, 2025. Should additional clinical documentation be required to support this determination, MUSC Health is prepared to provide the complete encounter record upon request. This appeal is being submitted in advance of the October 21, 2025, appeal deadline.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

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Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record